



DPI Safeguards Accelerator 2026

Safeguards for community health data tools in Uganda

Consent, offline inclusion, and a complaint path for FairBanks FCHIP

Your health, our mission.

Stage 1 answer bank for Airtable · Source check 2026-07-29

Field	Value
Legal name	FAIRBANKS MEDICAL CENTRE LIMITED
Known as	FairBanks Medical Centre
Country	Uganda
Pathway	NGO form — community-based health operator (private company)
Track	Issue-specific safeguards
Ask	USD 70,000 / 9 months
Deadline	30 July 2026
Contact	Racheal Nabukeera, Managing Director and Co-founder
Email / phone	info@fairbanksmedicalcentre.com · +256 772 849 258
Counterpart	Kampala Capital City Authority (KCCA) — Kawempe Division health structures
Address	Tirupati Road, Kyebando Kisalosalalo, Kawempe Division, Kampala

1. Call facts

Item	Detail
Programme	DPI Safeguards Accelerator (first cohort)
Stewards	UN ODET and UNDP; with 50-in-5, OGP, and Co-Develop
Pathway	NGO pathway form (Uganda only) — community-based health operator
Deadline Stage 1	30 July 2026
Stage 2 (if invited)	Full proposal + government letter by 15 Aug 2026
Ask	USD 70,000 over 9 months
Track	Issue-specific safeguards work
Government counterpart	Kampala Capital City Authority (KCCA) — Kawempe Division health structures
Submit via	https://airtable.com/appcltTa6tzGwskOA/pagJNFsEqox4M5WAU/form

Official pages always win if this pack and the form differ. Stage 1 is a concise screening form — not a full proposal.

2. Applicant facts (from company-docs)

Stage 2 still needs the KCCA support letter and incorporation papers. We do not invent NGO status.

Item	Detail
Legal form	FAIRBANKS MEDICAL CENTRE LIMITED — private company limited by shares (No. 80020003843337). Not an NGO; apply as community-based health operator.

Lead contact	Racheal Nabukeera, Managing Director and Co-founder · info@fairbanksmedicalcentre.com · +256 772 849 258
Address	Tirupati Road, Kyebando Kisolosalo, Kawempe Division, Kampala
Government counterpart	Kampala Capital City Authority (KCCA) — Kawempe Division health structures
Stage 2 letter	No MoU yet. First Stage 2 task: written support from KCCA / Kawempe Division health.
Stage 2 legal proof	Certificate of Incorporation + UMDPC 2026 licence from company-docs/

3. Why this fits

Community and clinic health data are going digital in Kampala, but consent, offline access, and a simple way to challenge mistakes are still thin — so trust can fail just as the tools scale.

We will embed consent rules, offline-friendly inclusion, and a community grievance path into FairBanks' live FCHIP health data platform in Kawempe, with KCCA Division health and VHTs at the table.

- Government link: Named counterpart: KCCA / Kawempe Division health. Stage 2 letter is the next step.
- Community role: VHTs, households, CHIS groups, and GeriCare clients help design and test — ongoing.
- Clear outputs: Six field assets in nine months, used inside FCHIP and Community Reach.
- Honest fit: Licensed clinic, working digital health MVP, peri-urban Kampala catchments.

Visual model for partners: Community Reach cascade (concept_improved) with FCHIP on Data & Feedback — safeguards close the loop: learn, improve, and serve again.

4. Stage 1 — Airtable answer bank

Paste or adapt these blocks into the NGO pathway form. Field labels may vary slightly; match meaning to the official Stage 1 topics listed on the Accelerator page.

4.1 Organisation, country, and DPI rollout

FAIRBANKS MEDICAL CENTRE LIMITED is a Uganda private company limited by shares (Company No. 80020003843337, incorporated 4 October 2022). We run a UMDPC-licensed medical centre and pharmacy on Tirupati Road, Kyebando–Kisolosalo, Kawempe Division, Kampala. Through FairBanks Community Reach we work with community members, Village Health Teams and community health workers, schools and local groups in Bukoto, Kyebando, Kisaasi, Kamwokya, Kikaaya and nearby areas. Care runs from home and community outreach into our medical centre. We also run GeriCare for older people, maternal and child support, chronic-disease screening, and a community health insurance scheme (CHIS) so families can share the cost of care. The digital piece is FCHIP — the FairBanks Community Health Intelligence Platform. It is already a working MVP. Frontline workers can capture household and outreach data on phones even when the network is weak. The platform brings community signals together with facility records and secure data links so existing clinic systems can send approved clinical data into FCHIP without forcing clinics to throw away the software they already use. In this Accelerator we are not selling software. We are putting practical safeguards into that live digital health rollout while it is still young.

4.2 Active DPI initiative being supported

The DPI we support is sectoral digital health infrastructure for community-to-facility data exchange in Uganda. In practice that is FCHIP: last-mile VHT and community health worker data, school and outreach signals, FairBanks Medical Centre workflows, and narrow links to existing clinic records systems. Uganda's digital health tools are growing — continuity of care, data exchange, and dashboards that guide outreach. People feel that in their homes and at the clinic desk. Our focus is trust: clear consent, offline access for people with weak networks, and a real way to challenge mistakes before the platform scales.

4.3 Government counterpart agency

Our named counterpart is Kampala Capital City Authority (KCCA) — Kawempe Division health structures. Our facility is licensed and operates in Kawempe Division. Community Reach and VHT-linked work sit in the same city health geography. We already engage the public health system as a licensed private provider (UMDPC licence current for 2026) and through frontline VHT relationships. We do not yet have a signed MoU or commitment letter for this Accelerator. If longlisted, Stage 2 requires written support. Getting that letter is our first Stage 2 task, led by Racheal Nabukeera.

4.4 Track focus

Issue-specific safeguards work. Over nine months we will put three concrete safeguards into the live FCHIP deployment: (1) consent and responsible data sharing — clear household consent, purpose limits, least-privilege access for any facility data link, and a simple audit trail; (2) offline and inclusion — tools that work without steady internet, design for low literacy, and reach to women, older people, people with disabilities, and poorer urban households; (3) grievance and recourse — a named route to ask about data, request a correction, or raise a concern about an alert, with someone responsible and a response time. We start with a short risk map of the FCHIP data path so the three measures sit in the right order.

4.5 Why this deployment moment is critical

FCHIP is past the blank-page stage. The MVP works. We are moving into real use with VHTs and our facility team. That is when bad habits stick — weak consent, open data pipes, no way for a mother in Kyebando to challenge a wrong record. If we wait until the platform is finished, fixing trust later will be harder. Putting safeguards in now means consent, access rules, offline access, and a complaint path ship with the same workflows we are validating in Bukoto, Kyebando, Kisaasi, Kamwokya and Kikaaya. Uganda needs examples grounded in a real clinic and real communities, not only frameworks on paper.

4.6 Expected tangible outputs (6–9 months)

By the end of nine months we will deliver and use: (1) a short safeguards risk-and-gap note for the FCHIP community–facility data path; (2) a consent and data-sharing protocol used in VHT capture and any facility records link; (3) an inclusion checklist and field playbook for offline use and underserved groups; (4) a live grievance pathway with named owners, contact channel, and response times, tested with community members; (5) a small working group that meets monthly — FairBanks clinical and Community Reach leads, VHT representatives, a facility contact, and a KCCA/Kawempe liaison once confirmed — with simple meeting notes; (6) a two-to-four page learning note for the DPI Safeguards Resource Hub. We will judge success by use in the field, not by documents sitting in a folder.

4.7 Coordination model

FairBanks: Racheal Nabukeera (Managing Director and Co-founder) holds overall accountability. Clinical and Community Reach coordinators run field tests. A technical lead wires consent prompts, access controls, and offline behaviour into FCHIP. A data-protection lead (named in Stage 2) owns the protocol and the grievance log. Government: KCCA / Kawempe Division health focal point reviews priorities, joins the monthly working group when available, and is the path for the Stage 2 support letter. Civil society and affected users: VHTs from our catchments co-write consent wording; community members — including women, older people, and GeriCare clients — help test offline tools and the complaint path; CHIS group representatives speak for families who pool money for care; local champions help check whether the grievance route is reachable. This is ongoing work, not a one-day workshop. Rhythm: kick-off in month 1; monthly working-group check-ins; community listening each quarter; end-of-cycle walkthrough of the three safeguards in live use.

4.8 Indicative budget and duration

Duration: 9 months. Request: USD 70,000. Rough split: staff time, VHT and community stipends, and safeguards facilitation USD 28,000; technical work on consent flows, access controls, offline UX, and grievance tooling USD 21,000; workshops, local Kampala travel, translation and printing USD 10,500; monitoring, documentation, learning note and cohort participation USD 10,500. This speeds up safeguards inside work we already run. It does not replace clinic income or Community Reach operating costs.

5. Indicative budget (Stage 1 range)

Line	USD
Staff time, VHT/community stipends, safeguards facilitation	28,000
Technical work (consent, access controls, offline UX, grievance tooling)	21,000
Workshops, local travel, translation, printing	10,500
Monitoring, documentation, learning note, cohort participation	10,500
Total	70,000

Refine line items, quotations, and any co-funding before Stage 2. Catalytic funds accelerate safeguards inside an active deployment.

6. How this maps to Universal DPI Safeguards themes

Accelerator theme	FairBanks / FCHIP response
Responsible data use / consent / sharing	Consent protocol + least-privilege EMR/HMS APIs + audit trail
Inclusion / offline access	Offline CHW tools; low-literacy UX; reach to vulnerable groups
Grievance / recourse	Named pathway for data and alert-related concerns
Multistakeholder governance	Working group: government liaison, VHTs, community, facility
Sectoral DPI (digital health)	Community health data exchange as public-interest infrastructure

7. Stage 2 preparation (if longlisted)

- Month-by-month activity plan with owners
- Final budget with quotes and salary scales
- Certificate of Incorporation + UMDPC licence as legal proof
- Written support letter from KCCA / Kawempe Division health
- Named VHT and community roles and meeting dates
- Simple data-protection and safeguarding note for health data
- Risks: privacy, exclusion, wrong alerts, weak adoption — and how we handle them

- Learning note plan for the DPI Safeguards Resource Hub

Advancement to Stage 2 does not guarantee funding. Final awards follow fiduciary and programmatic clearance.

8. Submit checklist (before 30 July)

- Use legal name FAIRBANKS MEDICAL CENTRE LIMITED on the form.
- Paste answers from stage1_airtable_answers.md (match field meaning).
- Open the NGO Airtable form (use a network that allows Airtable if blocked).
- Save confirmation or screenshot after submit.
- Same day: draft the KCCA / Kawempe Division support-letter request.

9. Official sources

Source	URL
Accelerator page	https://www.dpi-safeguards.org/accelerator
DPI Safeguards home	https://www.dpi-safeguards.org/
NGO Stage 1 Airtable form	https://airtable.com/appcltTa6tzGwskOA/pagJNFsEqox4M5WAU/form
fundsforNGOs programme summary	https://www2.fundsforngos.org/construction/safeguards-accelerator-supporting-safe-and-inclusive-digital-public-infrastructure/

Generated 2026-07-29. Follow <https://www.dpi-safeguards.org/accelerator> if anything conflicts.